

Identifying Counties with Overlapping Postpartum Medicaid Access Barriers

Findings from the Postpartum Medicaid Access Barrier Index

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Research question: Where may postpartum Medicaid populations face overlapping administrative, clinical, and contextual access barriers, and which counties would be missed by a screen focused only on Medicaid office and hospital-based obstetric care gaps?

Why This Matters

Postpartum Medicaid access depends on more than formal eligibility. After delivery, people may need to maintain coverage, complete renewals, navigate documentation, find local assistance, and connect to postpartum or maternity-related care. Counties with limited in-person Medicaid support, no hospital-based obstetric care, transportation barriers, digital access barriers, language access needs, disability-related support needs, and rural geography may create overlapping conditions that make coverage and care navigation harder.

A screen focused only on whether a county has both no Medicaid office and no hospital-based obstetric care captures the most visible infrastructure gaps, but it can miss counties where one infrastructure gap combines with substantial contextual burden. This brief asks what the full index adds beyond that narrower screen.

What the Index Found

The index did more than identify counties with the most obvious infrastructure gaps. A **dual-gap infrastructure screen** flags counties only when they have both no Medicaid office and no hospital-based obstetric care. The full index also identified counties where one infrastructure gap, or no direct infrastructure gap, overlapped with substantial contextual access burden.

The key added-value finding: 279 high/highest concern counties would have been missed by a dual-gap-only infrastructure screen.

Measure	Result
Counties classified as high or highest concern	665
Captured by dual-gap infrastructure screen	386 / 665, 58.0%
Missed by dual-gap infrastructure screen	279 / 665, 42.0%

These missed counties were not low-burden outliers. They had an average score of 6.46 and an average contextual support-burden score of 4.56. Most were nonmetro, and large shares had high poverty, high no-internet rates, high disability rates, or high no-vehicle household rates. This is the main reason to use the full index rather than a screen based only on the two infrastructure gaps.

What the Index Adds

The index does not only ask whether a county has both major infrastructure gaps. It distinguishes administrative access, clinical maternity infrastructure, and access-support burden.

In other words, the index helps confirm where additional barriers matter within counties that would otherwise appear less urgent under a dual-gap-only screen.

Score Design

The Postpartum Medicaid Access Barrier Index is a transparent county-level screening score. The theoretical score range is 0-11, and observed county scores ranged from 0 to 10.

Component group	Fields	Points
Infrastructure gaps	No Medicaid office in county; no hospital-based obstetric care	2 points each
Contextual flags	Poverty; no vehicle; no internet; limited English; disability; female ages 15-44; nonmetro county status	1 point each

Practical Review Sequence

- 1. Start with highest-concern counties.** Review the 56 highest-concern counties first, including the 11 counties with the maximum observed score of 10.
- 2. Review counties with both infrastructure gaps.** Counties with both no Medicaid office and no hospital-based obstetric care are the clearest infrastructure-gap counties.
- 3. Review one-gap plus high-context counties.** These counties may not appear in an infrastructure-only screen but still have multiple access-support barriers.
- 4. Review contextual-burden watchlist counties.** A smaller group reaches high concern through contextual burden without either direct infrastructure gap.

Applied Screening Rule

Among the 279 high/highest concern counties missed by the infrastructure-only screen, a rule requiring **one infrastructure gap plus at least four contextual flags** recovered 265 counties, or 95.0%, with 100.0% precision.

Geographic Pattern

High scores were concentrated in the South, parts of the Southwest, and Alaska. Mississippi had the highest average county score and the highest share of high/highest concern counties. Texas had the largest number of high/highest concern counties.

Bottom Line

The most obvious infrastructure gaps are only part of the access-burden picture. The index provides a transparent way to identify counties that may be overlooked by simpler Medicaid access screens and to show where additional barriers make those counties worth closer review.

Repository: <https://github.com/caroline-howard/postpartum-medicaid-access-barrier-index>